



Emergency Psychiatric Treatment

Policy code	SMHS: 18
Original endorsement date	<i>June 2003</i>
Date of current endorsement	<i>9 January 2026</i>
Functional sub group	Mental health
Summary	The South Metropolitan Health Service (SMHS) will provide emergency psychiatric treatment when clinically indicated and will carry this out in accordance with relevant legislation and standards of practice.
Policy owner	Executive Director Mental Health
Contact	SMHS Mental Health Policy Officer.
Review date	<i>9 January 2029</i>
NSHQHS Standards	Standard 1: Clinical Governance Standard 2: Partnering with Consumers Standard 5: Comprehensive Care Standard 6: Communicating for Safety Standard 8: Recognising and Responding to Acute Deterioration
Chief Psychiatrist Standards for Clinical Care	Standard 2 - Assessment Standard 4 - Consumer and carer involvement in individual care Standard 6 - Risk assessment and management Standard 7 - Seclusion and bodily restraint reduction
Status	<i>Active</i>

1. Background

1.1. About this document

The South Metropolitan Health Service (SMHS) will provide emergency psychiatric treatment (EPT) where clinically indicated and will carry this out in accordance with relevant legislation and standards of practice. The administration of EPT must comply with sections 202, 203 and 204 of the [Mental Health Act 2014](#) (MHA 2014).

Refer to the Chief Psychiatrist of Western Australia [Clinicians Practice Guide to the Mental Health Act 2014](#)

1.2. Key definitions

Term	Definition
Capacity	A person is determined to have capacity if they are capable of: • understanding the information being given, the nature of the decision and its consequences • retaining the information as necessary to make a decision • using and weighing the information in the decision-making process • communicating their decision in some way.
Consent to (medical) treatment	In the context of health care, consent is a person's agreement that a health practitioner can proceed to perform a specific proposed treatment.
Emergency Psychiatric Treatment (EPT)	An intervention of last resort that may include the provision of: • medication or • medication in conjunction with restraint and/or seclusion. EPT can be frightening, distressing, and potentially traumatic for people and as per the MHA 2014 s.202, EPT must only be provided to: • save the person's life; or • prevent the person from behaving in a way that is likely to result in serious physical injury to the person or another person.
Non-authorised healthcare setting	Any healthcare setting that is not an authorised hospital.
Person centred care	An approach to care delivery where the person is treated respectfully as an individual human being, and not just as an illness or condition to be treated. It involves seeking out and understanding what is important to the person, their families, carers and support people, fostering trust and establishing mutual respect. It also means working together to share decisions and plan care.
Personal Support Person (PSP)	Can be any of the following people who are supporting someone who is experiencing mental illness – close family member, carer, their nominated person; the parent or guardian of a child or any enduring guardian of an adult MHA 2014 s.7. Some consumers may use the term kin to refer to a family member.

Trauma informed care

Care that recognises the possibility of the ongoing effects of past and current trauma in the lives of everyone seeking support and how this can affect the way people seek help. Trauma informed care emphasises autonomy, collaboration, self-determination and strengths-based approaches.

2. Policy Statement

SMHS mental health staff will:

- provide EPT only when absolutely necessary to save life or prevent serious physical injury
- not give EPT because the clinician believes it would be beneficial to commence treatment before the person is examined or made involuntary
- actively minimise harm, fear, and re-traumatisation
- communicate openly and respectfully with the person throughout EPT where possible
- support the person after EPT to understand what occurred and to reduce the likelihood of future EPT.

As per section 202 and 210 of the [MHA 2014](#) EPT does not include:

- electroconvulsive therapy (ECT)
- psychosurgery
- prohibited treatments (deep sleep therapy, insulin coma therapy or insulin sub coma therapy)
- treatment for a medical condition. Refer to the [Guardianship and Administration Act 1990](#).

EPT may be provided to any person including:

- voluntary consumers in an authorised hospital including persons referred on a Form 1A Referral for examination by a psychiatrist
- a voluntary person who has not been referred in a non-authorised setting e.g. Emergency Department or general hospital ward who meets the criteria for EPT.

2.1. Person centred care

Where EPT is clinically indicated, staff will provide care in a way that recognises the person's humanity, distress, and right to dignity.

Staff will, wherever possible:

- treat the person as an active participant in their care, even when consent cannot be obtained
- provide trauma informed care recognising that fear, agitation, withdrawal, or resistance may be trauma responses
- communicate clearly, calmly, and respectfully before, during, and after EPT
- explain what is happening and why, using language the person can understand
- offer reassurance and grounding support where practicable
- provide care in the least intrusive manner possible and for the shortest duration required
- ensure privacy, cultural safety, gender safety, and respect for spiritual beliefs

- for Aboriginal consumers ensure care is culturally secure and respects Aboriginal cultural values, spiritual beliefs and connection to country
- engage Aboriginal Mental Health Liaison or Aboriginal Health Liaison early in the process where appropriate.

The absence of consent does not remove the requirement to treat the person with compassion and respect.

2.2. Carers and other personal support persons

The [MHA 2014](#) acknowledges and respects the role of carers and other support persons in the provision of treatment, care and support to a person who has a mental illness. Wherever possible PSPs should be engaged and supported in treatment planning decisions.

The consumer's carer / PSP or Aboriginal Mental Health Liaison / Aboriginal Health Liaison should be engaged to support the consent process and the provision of EPT where appropriate and practicable. Staff will recognise the role of extended family, kinship networks, elder Aboriginal communities and Aboriginal healers and involve them when appropriate and practicable.

Refer to:

- [WA Carers Recognition Act 2004](#)
- [SMHS Rights of Carers and other Personal Support Persons](#)

2.3. Consent

Although EPT may be provided without consent when clinically indicated it is considered best practice for the clinician to first seek consent from the person. Where a person is offered treatment and gives their informed consent, it is not deemed to be EPT.

Consent is valid if given voluntarily, is informed, the person is an adult and has capacity. A failure to resist treatment does not constitute consent to treatment and clinicians must regard a person's wishes if practicable.

To ensure informed decision making the clinician will:

- provide relevant information regarding the proposed treatment to the consumer that is appropriate in terms of the consumers language and communication needs, health literacy, and cultural needs
- use culturally adapted communication tools to explain EPT when available
- ensure that information provided includes material risks and other treatment options (including having no treatment) in a way the consumer can understand, as relevant
- provide the consumer the opportunity to ask questions and be heard and afforded time and support to understand the information presented where practicable.
- ensure all consumers and carers / PSP who need assistance with standard English have access to appropriate interpreter services
- when possible, with the support of Aboriginal Mental Health Liaison or Aboriginal Health Liaison for Aboriginal consumers.

Refer to:

- [DoH Consent to Treatment Policy](#)
- [SMHS Consent to Treatment Policy](#)

Where the criteria for EPT is met a medical practitioner may provide a person who is voluntary or referred with EPT without informed consent.

2.4. Restrictive Practices

The [MHA 2014](#) provisions for EPT do not allow for the use of reasonable force to administer medication. Where EPT occurs within an authorised hospital and the person requires restraint for the treatment to be given that is bodily restraint and the MHA 2014 forms 10A – 10I Bodily Restraint must be completed in addition to the MHA 2014 form 9A Record of EPT.

Where EPT occurs within an authorised hospital and the person is subsequently secluded the MHA 2014 forms 11A – 11G Seclusion must be completed in addition to the MHA form 9A.

Refer to:

- [MHA 2014](#) Part 14 Division 5 & 6
- [FSFHG Restrictive Practices](#)
- [RkPG Restraint and Seclusion: Reducing Restrictive Interventions](#)

Where EPT occurs within a non-authorised healthcare setting and the person requires restraint for the treatment to be given refer to:

- [DoH Use of Restrictive Practices in Non-Authorised Healthcare settings policy](#)
- [FSFHG Restraint in Non-Mental Health Areas](#)
- [RkPG Restraint and Seclusion: Reducing Restrictive Interventions](#)
- [PHC Restraint in non-mental health areas](#)

2.5. Post EPT

As soon as practicable after EPT, the treating team will offer the person:

- a clear explanation of what occurred and the reasons for EPT
- an opportunity to ask questions and express how the experience affected them
- acknowledgement that the experience may have been frightening or distressing.

Where appropriate, staff will:

- offer emotional support and grounding following EPT
- involve carers or PSP in post-EPT discussions
- review triggers, early warning signs, and alternative strategies to reduce the likelihood of future EPT
- update Treatment Support and Discharge Plan and any safety plan in collaboration with the person.

2.6. Documentation

A medical practitioner who provides the EPT must as soon as practicable document that EPT was provided and complete MHA 2014 form 9A which includes:

- the name of the person provided with the treatment
- the name and qualification of the practitioner who provided the treatment
- the names of any other people involved in providing the treatment
- the date, time and place where the treatment was provided
- particulars of the circumstances of why the EPT was required
- particulars of the actual treatment provided such as the type of medication given.

A copy of the MHA 2014 form 9A must be:

- given to the person who received EPT
- made available in the person's medical record
- made available in PSOLIS
- submitted to the Chief Psychiatrist
- submitted to the Mental Impairment Review Tribunal where the person is a detainee under the [Criminal Law \(Mental Impairment\) Act 2023](#).

3. Roles and Responsibilities

3.1. Executive

SMHS MH Site Executives will ensure that managers and supervisors are aware of the legislative requirements and standards of practice relating to the provision of EPT.

3.2. Managers/Supervisors

Managers and Supervisors will develop and monitor strategies to maximise the safety of consumers, PSPs, staff and the public and provide staff with appropriate training and development that includes:

- trauma informed and person-centred care including Aboriginal Person-Centred Care training
- Aboriginal cultural safety and culturally secure practices related to EPT
- safe application of de-escalation techniques
- minimising the risk of violent or aggressive incidents.

3.3. All staff

Staff will conduct early assessment and timely reassessment of consumers who may be at risk of becoming violent or aggressive.

Staff are to ensure that they are familiar with what is required of their role in the provision of EPT.

4. Compliance monitoring

Each SMHS MH site Service Director/Co-Director is to ensure compliance with this policy. Compliance monitoring methods may include the following:

- ensuring staff in their area are made aware of and comply with this policy
- ensuring staff providing EPT receive required training
- review of all EPT provided including Aboriginal liaison involvement for cultural safety measures.

5. Legislation

There are legislative obligations relating to this policy for which a statutory penalty is imposed for a breach.

- [Mental Health Act 2014](#)
- [Mental Health Regulations 2015](#)
- [Criminal Law \(Mental Impairment\) Act 2023](#)
- [Carers Recognition Act 2004](#)
- [Guardianship and Administration Act 1990](#)

6. Related Documents

6.1. WA Health mandatory policies.

- [MP 0175/22 Consent to treatment policy](#)
- [DoH Consent to treatment procedure](#)
- [MP 0181/24 Safety planning for mental health consumers](#)
- [MP 0186/24 Use of restrictive practices in non-authorised healthcare settings](#)
- [WA Aboriginal Health and Wellbeing Framework 2015–2030](#)

6.2. SMHS policies

- [Consent to Treatment](#)

6.3. Other documents

- [National Aboriginal and Torres Strait Islander Health Plan 2021 – 2031](#)
- [Chief Psychiatrists Standards for Clinical Care](#)
- [Chief Psychiatrists Clinicians guide to the Mental Health Act 2014](#)
- [Chief Psychiatrists Guideline Use of detention and restraint in non-authorised healthcare settings.](#)

6.4. Document Control

Version	Published date	Effective from	Review date	Effective to*	Amendment (s)
GP V6	13 January 2026	9 January 2026	9 January 2029	January 2029	Triennial Review

6.5. Approval

Approval by	Executive Director, Mental Health
Approval date	22 December 2025

This document can be made available in alternative formats on request.

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